

GA HEALTHCARE TRAINING & CONSULTING

Building Skills. Changing Lives. Advancing Healthcare.

NURSE AIDE TRAINING PROGRAM (NATP)

BLENDed LEARNING TRACK

POLICY & PROCEDURE MANUAL

Total Program Hours: 98

Online Theory (Self-Paced): 40 hrs | In-Person Skills Laboratory: 34 hrs | Clinical Orientation: 24 hrs

Course Text: Nursing Assistant Care: The Basics, 6th Edition (Jetta Fuzy, Hartman Publishing; ISBN 978-1-60425-141-8)

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Founder & Chief Executive Officer

Version 7.0 — Blended Learning Track

Effective August 2026 · Reviewed and Revised Annually

DOCUMENT CONTROL & REVISION HISTORY

Version	Date	Prepared / Approved By	Summary of Changes
6.0	Prior release	Program Director	Virtual/Hybrid Track manual, 98 hours, Hartman 6th Edition alignment.
7.0	August 2026	Y. Nozile, PhD, RN — Founder & CEO	Full revision to Blended Learning terminology; competency evaluation vendor identified as Credentia and the Georgia Nurse Aide Registry attributed to Alliant Health Solutions; organizational abbreviation corrected to GAHCTC; grading standards standardized at 80% written / 100% skills; assessment architecture (module quiz, chapter test, in-person final) documented; new sections added for program approval status, admission and enrollment, technology requirements, nondiscrimination and accommodations, student records, health and safety, program completion, and instructor qualifications; student learning outcomes rewritten. Clinical policy expanded with schedule and preconference, site conduct, incident procedure, and a \$25.00 liability insurance requirement. See Appendix A.

This manual is a controlled document. The Program Director maintains the master copy. Superseded versions are archived and retained for a minimum of three (3) years. Students, instructors, and clinical partners are notified in writing of any substantive revision.

PROGRAM APPROVAL STATUS — READ BEFORE DISTRIBUTION

The GA Healthcare Training & Consulting Nurse Aide Training Program (blended learning track) has an application pending with the Georgia Department of Community Health (DCH). Until written approval is received from DCH, this program must not be described in any manual, website, advertisement, enrollment agreement, or verbal representation as approved, accredited, certified, or state-licensed. This manual describes the program as designed and submitted for approval. Upon receipt of written DCH approval, the Program Director will update this notice, insert the assigned program approval number and effective date, and reissue the manual as Version 7.1.

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Competency Evaluation Vendor	Credentia — NNAAP®, under contract with Georgia DCH
Georgia Nurse Aide Registry	Administered by Alliant Health Solutions for Georgia DCH
Regulatory Authority	Georgia Department of Community Health (DCH)

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SECTION 1: PROGRAM OVERVIEW

1.1 Mission Statement

GA Healthcare Training & Consulting (GAHCTC) is committed to delivering high-quality, evidence-based nurse aide education in a blended learning format that prepares competent, compassionate, and ethical healthcare professionals. The GAHCTC Nurse Aide Training Program (NATP) equips participants with the knowledge, clinical skills, and professional behaviors required to provide safe, effective, person-centered care across diverse healthcare settings, while accommodating the flexibility adult learners need through self-paced online instruction paired with supervised in-person practice.

1.2 Vision

To be a premier blended learning healthcare training institution in Georgia, recognized for workforce excellence, educational innovation, accessibility, and a steadfast commitment to advancing the nursing profession and the communities we serve.

1.3 Core Values

- Compassion — care is delivered with dignity, empathy, and respect for every resident, family, and colleague.
- Excellence — instruction, evaluation, and clinical practice are held to measurable, evidence-based standards.
- Integrity — honesty, accountability, and regulatory compliance govern every aspect of program operation.

1.4 Program Philosophy

This program is grounded in andragogical principles (Knowles, 1980), recognizing that adult learners are self-directed, experienced, goal-oriented, and motivated by real-world application. Instruction is learner-centered, culturally responsive, and aligned with federal and Georgia state regulatory standards. The program uses Nursing Assistant Care: The Basics, 6th Edition (Jetta Fuzy, Hartman Publishing) as the primary instructional resource, delivered through the GAHCTC TalentLMS online campus, supplemented by in-person skills laboratory sessions at the GAHCTC training facility and supervised clinical orientation in a licensed nursing home.

1.5 Program Purpose

The GAHCTC Nurse Aide Training Program prepares students to meet state and federal requirements for nurse aide certification in Georgia. Upon successful completion of all approved program hours, participants are eligible to sit for the Georgia Nurse Aide Competency Evaluation (NNAAP®, administered by Credentia) and, upon passing both the written/oral and clinical skills components, to be listed on the Georgia Nurse Aide Registry administered by Alliant Health Solutions.

1.6 Program Approval Status

GAHCTC has submitted an application to the Georgia Department of Community Health for approval of this Nurse Aide Training Program. Approval is pending. GAHCTC does not represent, and will not permit any employee, contractor, instructor, or marketing material to represent, that this program is approved, accredited, certified, or state-licensed until written approval is received from DCH. Prospective students may

join an interest list; enrollment agreements will not be executed and tuition will not be collected for the NATP until approval is granted and this manual is reissued with the approval number recorded in the Document Control table.

1.7 The Blended Learning Model Defined

GAHCTC uses the term blended learning — not "hybrid" or "virtual" — to describe this track. Blended learning at GAHCTC means a single, integrated 98-hour program in which theory instruction is completed online at the student's own pace, while every psychomotor skill and all resident contact occur in person under the direct supervision of a qualified Registered Nurse instructor. The online and in-person components are not alternatives to one another; both are mandatory, sequenced, and separately documented.

Component	What It Is	What It Is Not
Online theory (40 hrs)	Self-paced TalentLMS modules with assigned readings, recorded instruction, knowledge checks, module quizzes, and chapter tests. Completion is time-tracked per student.	Not a substitute for skills laboratory hours. Not a correspondence course. Not ungraded self-study.
Skills laboratory (34 hrs)	On-site, instructor-led demonstration, return demonstration, and competency check-off at the GAHCTC training facility. On-site check-in required.	Not deliverable by video, simulation-only, or remote proctoring. Not waivable for prior experience.
Clinical orientation (24 hrs)	Supervised direct resident care in a licensed nursing home under a signed affiliation agreement.	Not observational shadowing. Not employment. Not completed in a home care, hospital, or assisted living setting.

1.8 Program Structure at a Glance

Component	Hours	Delivery Format
Online Theory Instruction (Self-Paced)	40	Online — GAHCTC TalentLMS
In-Person Skills Laboratory (On-Site Check-In)	34	In-Person — GAHCTC Skills Lab
Clinical Orientation (Nursing Home Facility)	24	In-Person — Affiliated Clinical Site
TOTAL PROGRAM HOURS	98	Blended Learning

Cohort dates, module release schedules, skills laboratory check-in dates, and clinical rotation dates are published on the GAHCTC TalentLMS campus and in the cohort calendar. They are not included in this manual because they vary by cohort.

1.9 Program Goals

- Deliver a minimum of 98 instructional program hours — 40 online theory, 34 in-person skills laboratory, and 24 supervised clinical orientation — meeting or exceeding the Georgia DCH minimum of 85 hours for nurse aide training programs.
- Develop foundational knowledge, psychomotor competence, and professional values using the Hartman 6th Edition curriculum delivered through a blended learning model.
- Foster an inclusive, adult-centered learning climate that supports diverse learners, including those balancing employment, family responsibilities, and education.
- Maintain full regulatory compliance with Georgia DCH rules, 42 CFR §§ 483.150–483.158, and CMS standards for nurse aide training and competency evaluation programs.

- Achieve a minimum 80% first-attempt pass rate on the Georgia Nurse Aide Competency Evaluation.
- Prepare graduates for employment in long-term care, home health, and hospital settings, and for continued advancement along the nursing career pathway.

SECTION 2: REGULATORY COMPLIANCE & LEGAL FRAMEWORK

2.1 Federal Requirements — OBRA '87 and 42 CFR §§ 483.150–483.158

The Omnibus Budget Reconciliation Act of 1987 (OBRA '87), codified at 42 CFR §§ 483.150–483.158, established the federal framework for all Nurse Aide Training and Competency Evaluation Programs (NATCEPs). The GAHCTC blended learning NATP is designed to operate in full compliance with all applicable federal and Georgia state requirements, including:

- Georgia-approved nurse aide training programs must provide a minimum of 85 hours of instruction divided among classroom/online theory, laboratory, and supervised clinical practice, with a minimum of 24 hours dedicated to supervised clinical orientation in a nursing home. The GAHCTC blended track exceeds this minimum with 98 total program hours.
- A minimum of 16 hours of training in the first six subject areas of Part I (I.A through I.F) must be completed before any direct contact with a resident. At GAHCTC these 16 hours are delivered online and verified in TalentLMS before a student may check in for any skills laboratory session involving resident-care simulation.
- The required hours must cover the full federal and state core curriculum as defined at 42 CFR §§ 483.150–483.158, together with all Georgia requirements for supervised clinical orientation.
- Competency evaluation is administered by a state-contracted vendor and consists of a written (or oral) knowledge examination and a clinical skills demonstration.
- Individuals listed on a nurse aide registry with a substantiated finding of resident abuse, neglect, mistreatment, or misappropriation of resident property are prohibited from employment as a nurse aide.
- All theory and skills instruction is conducted under the supervision of a Registered Nurse who meets Georgia instructor qualification requirements.
- Records and documentation — including online completion logs, attendance records, skills checklists, and clinical evaluations — are maintained as required by CMS and Georgia DCH.

2.2 Georgia DCH Requirements — Blended Learning Track

All GAHCTC program operations are designed to comply with the Georgia Department of Community Health rules governing nurse aide training and competency evaluation programs, including:

- Program approval and annual renewal with Georgia DCH, inclusive of the blended learning delivery model and any subsequent modification to delivery format, hours, faculty, or clinical sites.
- Curriculum content addressing all required Parts I through VII, inclusive of every sub-section, as organized in this manual and in the GAHCTC blended lesson plans.
- Online theory instruction delivered via TalentLMS, with per-student module completion, time-on-task, quiz scores, and test scores documented and exportable for audit.
- In-person skills laboratory sessions conducted at the GAHCTC training facility with documented on-site check-in and instructor-initialed skills check-off.
- Clinical orientation conducted in a licensed nursing home / long-term care facility holding a current signed affiliation agreement with GAHCTC.
- Instructor qualifications: a Registered Nurse licensed in Georgia with a minimum of two (2) years of nursing experience, including at least one (1) year of experience in long-term care.
- Maintenance of student records, including TalentLMS completion records, for a minimum of three (3) years.

- Criminal background check clearance and tuberculosis screening documentation on file for every student prior to clinical placement.

2.3 Pre-Contact Training Requirement — Minimum 16 Hours (Part I, Subjects A–F)

Federal regulation and Georgia DCH require that a minimum of 16 hours of instruction in the six subject areas below be completed before a participant may have direct contact with a resident or participate in resident-care simulation. At GAHCTC these hours are delivered online through TalentLMS and must be verified by the Program Director before skills laboratory check-in is authorized.

Part	Subject Area (42 CFR §§ 483.150–483.158)	Online Hours	Textbook Reference
I.A	Communication and Interpersonal Skills, including Stress Management and Chain of Command	2.5	Hartman Basics 6e — Ch 2
I.B	Infection Control — Chain of Infection, Standard and Transmission-Based Precautions, PPE, Aseptic Technique	2.5	Hartman Basics 6e — Ch 2
I.C	Safety and Emergency Procedures, Foreign Body Airway Obstruction, Body Mechanics	2.5	Hartman Basics 6e — Ch 2
I.D	Promoting Residents' Independence — Restorative Nursing Principles and Self-Care	2.5	Hartman Basics 6e — Ch 1, Ch 3
I.E	Respecting Residents' Rights (OBRA '87) — Privacy, Dignity, Ombudsman, Rights Violations	2.5	Hartman Basics 6e — Ch 1, Ch 3
I.F	Legal and Ethical Behavior and Scope of Practice — Negligence, HIPAA, Ethical Principles, Mandatory Reporting	2.5	Hartman Basics 6e — Ch 2, Ch 3, Ch 5
—	Part I Cumulative Review and Formative Assessment (required before skills laboratory check-in)	1.0	—
SUBTOTAL	Part I — Online Pre-Contact Training (required BEFORE in-person skills laboratory attendance)	16.0	—

Each of the six Part I subjects is allocated 2.5 hours of online content, totaling 15 hours. The remaining hour consists of structured cumulative review and a formative assessment across all six subjects, bringing the total to the 16 online hours required before any resident contact or resident-care simulation.

Page-level textbook references for each sub-unit are maintained in the GAHCTC Core Curriculum Required Skills Checklist and the Blended Course Map, which are the controlling documents for page citations. This manual cites chapters only, so that page corrections in the source documents do not create conflicts here.

2.4 Hour Distribution Summary — 98-Hour Blended Program

Component	Hours	Delivery Location
Online Theory Instruction — Parts I–VII (self-paced TalentLMS modules, knowledge checks, quizzes, chapter tests)	40	Online — GAHCTC TalentLMS
In-Person Skills Laboratory — demonstration, return demonstration, and skills check-off (on-site check-in required)	34	In-Person — GAHCTC Skills Lab
Clinical Orientation — supervised direct resident care	24	In-Person — Affiliated Nursing Home
TOTAL PROGRAM HOURS	98	Blended Learning

This 98-hour structure exceeds the Georgia DCH minimum of 85 program hours. Sixteen of the 40 online hours satisfy the federal and state 16-hour pre-contact training requirement; the remaining 24 online hours deliver Parts II–VII theory content.

2.5 Clinical Orientation Requirements

- All supervised clinical orientation hours must be completed in a licensed nursing home / long-term care facility, as required by Georgia DCH and 42 CFR §§ 483.150–483.158.
- A minimum of 24 hours of supervised clinical orientation is required for program completion.
- All 16 hours of Part I online pre-contact training must be completed and verified in TalentLMS before a student may attend any skills laboratory session involving resident-care simulation or participate in clinical orientation involving direct resident contact.
- Clinical orientation is supervised at all times by a qualified RN instructor or approved clinical preceptor.
- The student-to-supervisor ratio does not exceed 12:1 at any clinical site.
- Each clinical site must hold a current, signed affiliation agreement on file with GAHCTC before any student is placed.

2.6 Competency Evaluation and the Georgia Nurse Aide Registry

The Georgia Nurse Aide Competency Evaluation is the National Nurse Aide Assessment Program (NNAAP®) examination, administered by Credentia under contract with the Georgia Department of Community Health. The examination consists of two separately scored parts: a written (or oral) knowledge examination and a clinical skills evaluation. Candidates must pass both parts to be listed on the registry.

The Georgia Nurse Aide Registry is administered by Alliant Health Solutions on behalf of Georgia DCH. Nurse aides must complete a minimum of eight (8) hours of paid nursing-related work within each 24-month period to maintain active registry status. Registry access, verification, and renewal requirements are addressed in the final online module and reinforced during the final skills laboratory session.

Competency Evaluation Vendor

Credentia is the vendor for the Georgia NNAAP® competency evaluation, and Alliant Health Solutions administers the Georgia Nurse Aide Registry. Student handouts, enrollment materials, the website, and any DCH application enclosures must name Credentia and Alliant Health Solutions consistently with this manual. The Program Director verifies vendor and registry information against the current Credentia Georgia candidate handbook before each cohort.

2.7 Georgia CNA Scope of Practice

The program dedicates specific online instruction to the Georgia certified nurse aide scope of practice (Part I.F), including delegated versus non-delegated tasks, legal principles (negligence, malpractice, false imprisonment), HIPAA and protected health information, ethical principles (beneficence, nonmaleficence, autonomy, justice), and mandatory reporting obligations. Students are trained to recognize the limits of the nurse aide role and to operate strictly within the Georgia-defined scope of practice, escalating through the chain of command whenever a task falls outside that scope.

2.8 Nondiscrimination and Equal Opportunity

GAHCTC admits students of any race, color, national or ethnic origin, sex, age, religion, disability, marital status, veteran status, or any other characteristic protected by applicable federal, state, or local law, and extends to all students all rights, privileges, programs, and activities generally made available to students of the program. GAHCTC does not discriminate on any protected basis in the administration of admissions, instruction, evaluation, clinical placement, disciplinary action, or grievance procedures.

2.9 Accessibility and Reasonable Accommodations

GAHCTC provides reasonable accommodations to qualified students with documented disabilities, consistent with the Americans with Disabilities Act, provided the accommodation does not fundamentally alter the nature of the program or compromise resident safety. Students requesting accommodation should contact the Program Director in writing at the time of enrollment, or as soon as the need arises, and supply supporting documentation from a qualified professional.

- Online accommodations may include extended time on module assessments, alternative formats for written material, or captioned instructional media.
- Skills laboratory and clinical accommodations are evaluated individually. Because certified nurse aide practice requires the safe performance of specific psychomotor skills, accommodations cannot waive any required skill or reduce the required program hours.
- Accommodations for the state competency evaluation are requested separately and directly from Credentia at the time of examination registration; GAHCTC will supply supporting documentation on request but does not control that determination.
- All accommodation requests and determinations are documented and retained confidentially in the student record.

SECTION 3: ADMISSION, ENROLLMENT & TECHNOLOGY REQUIREMENTS

3.1 Admission Requirements

Applicants must meet all of the following criteria. Documentation is verified by the Program Director and retained in the student record.

- Valid, unexpired government-issued photo identification.
- Proof of high school graduation or equivalent — high school diploma, official transcript, GED, or a college transcript documenting at least two years of coursework toward a four-year degree.
- Satisfactory criminal background check. Felony convictions and offenses involving violence, abuse, neglect, exploitation, or crimes against a person or a child may disqualify an applicant from admission, clinical placement, or registry listing.
- Negative tuberculosis screening — PPD or QuantiFERON — dated within the period required by the assigned clinical facility.
- Ability to read, write, speak, and understand English at a level sufficient to follow instruction, document care, and communicate with residents and licensed staff.
- Physical ability to perform the essential functions of the nurse aide role, including standing for extended periods, bending, and assisting with resident transfers, with or without reasonable accommodation.

Clinical Clearance Deadline

Background check clearance and tuberculosis screening documentation must be complete and on file before a student may begin clinical orientation. A student whose documentation is incomplete on the first scheduled clinical day will be withdrawn from the clinical rotation and deferred to the next available cohort. Because clinical hours cannot be reduced or substituted, this deferral cannot be waived.

3.2 Enrollment Process

1. Submit a completed GAHCTC application and all required admission documentation.
2. Complete an admissions interview or orientation session with the Program Director or designee.
3. Execute the enrollment agreement, tuition and fee schedule acknowledgment, HIPAA confidentiality agreement, clinical practicum agreement, and this Policy & Procedure Manual acknowledgment.
4. Remit tuition in full, or receive written approval of a payment plan, prior to release of TalentLMS credentials.
5. Complete platform orientation in TalentLMS and confirm access to all Part I modules.
6. Complete Part I online pre-contact training (16 hours) and pass the Part I cumulative assessment before the first scheduled skills laboratory check-in.

3.3 Technology Requirements

Because 40 program hours are delivered online, each student is responsible for maintaining access to technology sufficient to complete coursework. GAHCTC does not supply devices or internet service. Students who cannot meet these requirements should discuss options with the Program Director before enrolling.

Requirement	Minimum Standard
Device	Laptop, desktop, or tablet capable of running a current web browser. A smartphone alone is not sufficient for assessments or document submission.

Requirement	Minimum Standard
Internet	Reliable broadband connection capable of streaming instructional video without interruption.
Browser	Current version of Chrome, Edge, Safari, or Firefox with cookies and JavaScript enabled.
Audio	Working speakers or headphones. A microphone and camera are required for any live session or virtual office hours offered by the instructor.
Documents	Ability to open PDF files and to upload assignment files or clear photographs of completed documents.
Email	A monitored personal email address. GAHCTC and TalentLMS notifications are sent to this address and are considered official notice.

3.4 Account Integrity

- TalentLMS credentials are issued to one individual and may not be shared, transferred, or used by any other person.
- Permitting another person to complete online modules, quizzes, or tests on your behalf is an academic integrity violation resulting in immediate dismissal.
- Students must report a suspected compromise of their account to the Program Director immediately.

SECTION 4: PROGRAM LEARNING OUTCOMES

Upon successful completion of the GAHCTC Nurse Aide Training Program, the graduate will be able to perform the following, as evidenced by online assessment scores, instructor-verified skills check-off, and clinical preceptor evaluation. These outcomes are aligned with OBRA '87, 42 CFR §§ 483.150–483.158, Georgia DCH requirements, and the Hartman Basics 6th Edition curriculum.

Communication and Professional Role

1. Describe the role, responsibilities, and legal scope of practice of the certified nurse aide in Georgia, and distinguish delegated from non-delegated tasks.
2. Use verbal, nonverbal, and therapeutic communication techniques with residents, families, and members of the interdisciplinary care team.
3. Apply the chain of command to report observations, concerns, and changes in resident condition to the supervising nurse in a timely and accurate manner.

Safety and Infection Control

4. Explain the chain of infection and demonstrate hand hygiene, standard precautions, transmission-based precautions, and correct donning and doffing of personal protective equipment without critical error.
5. Identify safety hazards in the long-term care environment and apply fall prevention, fire safety (RACE and PASS), emergency response, and evacuation procedures.
6. Demonstrate safe body mechanics and the management of foreign body airway obstruction in a conscious and an unconscious adult.

Assessment and Observation

7. Accurately measure, record, and report temperature, pulse, respirations, blood pressure, height, and weight, and identify values outside expected ranges.
8. Recognize and promptly report changes in a resident's physical condition, behavior, function, skin integrity, intake, and output.
9. Identify verbal and nonverbal indicators of pain and report them accurately to the supervising nurse.

Personal Care and Clinical Skills

10. Provide complete personal care — bathing, perineal care, grooming, oral hygiene, dressing, and toileting — while maintaining infection control, privacy, and dignity.
11. Assist residents safely with eating and hydration, recognize aspiration risk, and document intake, output, and refusals.
12. Perform transfers, positioning, turning, and gait belt ambulation using correct body mechanics and appropriate assistive or mechanical lift equipment.
13. Identify pressure injury risk factors and stages, and apply preventive skin care during routine care.

Psychosocial, Cognitive, and Restorative Care

14. Apply person-centered strategies to support residents experiencing depression, cognitive impairment, dementia, and behavioral and psychological symptoms of dementia, including validation, redirection, and de-escalation.
15. Support residents' emotional, spiritual, and cultural needs, and involve family members appropriately as part of the care team.
16. Apply basic restorative principles — range of motion, ambulation, assistive device use, bowel and bladder retraining, and prosthetic and orthotic care — to promote resident independence.

17. Provide comfort-focused care to residents at the end of life and perform postmortem care with dignity and cultural sensitivity.

Rights, Ethics, and Regulation

18. Identify residents' rights under OBRA '87 and describe the nurse aide's obligation to protect privacy, autonomy, personal property, and freedom from abuse, neglect, and unnecessary restraint.
19. Describe the facility grievance process and the role and authority of the Long-Term Care Ombudsman.
20. Apply HIPAA requirements to the handling of protected health information in verbal, written, and electronic form.
21. Recognize and report suspected abuse, neglect, or misappropriation of resident property in accordance with Georgia mandated reporter law.
22. Describe the federal and Georgia regulatory framework governing nurse aide competency, registry listing, renewal, and program approval, and complete the registration process for the Georgia Nurse Aide Competency Evaluation.

Instructor-facing objectives concerning instructional design, adult learning theory, and assessment construction — which appeared in this position in prior versions of the manual — have been relocated to Section 7.6, where they properly apply to faculty preparation rather than to student outcomes.

SECTION 5: PROGRAM CURRICULUM STRUCTURE

The GAHCTC blended learning NATP is organized across seven curriculum parts aligned with the federal and state core curriculum required under 42 CFR §§ 483.150–483.158 and delivered through self-paced online theory, in-person skills laboratory sessions, and supervised clinical orientation. Module release sequencing, laboratory check-in dates, and clinical dates are published on the TalentLMS campus for each cohort.

5.1 Online Theory Instruction — 40 Hours (Self-Paced)

Online instruction is delivered asynchronously and self-paced through TalentLMS. Each module contains recorded instruction, assigned readings from Hartman Basics 6th Edition, knowledge checks, and a graded module quiz. Each textbook chapter is followed by a graded chapter test. Students may work ahead within a released unit but may not skip a module, and the platform enforces sequential completion.

Part	Online Module / Topic	Hours	Textbook Reference
I.A	Communication and Interpersonal Skills, Stress Management, Chain of Command	2.5	Ch 2
I.B	Infection Control: Chain of Infection, Standard and Transmission-Based Precautions, PPE, Aseptic Technique	2.5	Ch 2
I.C	Safety, Emergency Procedures, Foreign Body Airway Obstruction, Body Mechanics	2.5	Ch 2
I.D	Promoting Residents' Independence	2.5	Ch 1; Ch 3
I.E	Respecting Residents' Rights (OBRA '87); Long-Term Care Ombudsman	2.5	Ch 1; Ch 3
I.F	Legal and Ethical Behavior and Scope of Practice: Negligence, HIPAA, Ethical Principles, Mandatory Reporting	2.5	Ch 2; Ch 3; Ch 5
—	Part I Cumulative Review and Assessment (required before skills laboratory check-in)	1.0	—
II	Assessment and Observation Skills: Vital Signs, Height and Weight, Resident Environment, Recognizing Abnormal Changes, Pain Recognition, End-of-Life and Postmortem Care	6.0	Ch 3; Ch 4; Ch 6; Ch 7
III	Personal Care and Clinical Skills: Bathing, Grooming and Mouth Care, Dressing, Toileting, Eating and Hydration, Feeding Technique, Skin Care and Pressure Injuries, Transfers, Positioning and Turning	6.0	Ch 6; Ch 8
IV	Mental Health and Social Service Needs	3.0	Ch 2; Ch 3; Ch 5
V	Cognitively Impaired Residents: Dementia and Alzheimer's Care, BPSD, Communication and Behavioral Approaches	3.0	Ch 2; Ch 3; Ch 5
VI	Basic Restorative Services: Ambulation, Gait Belt, Range of Motion, Assistive Devices, Bowel and Bladder Retraining, Prosthetics	3.0	Ch 3; Ch 6; Ch 8; Ch 9
VII	Residents' Rights: Privacy, HIPAA, Grievances, Ombudsman, Abuse, Restraint Avoidance; Cumulative Review and Assessment	3.0	Ch 1; Ch 3; Ch 7
TOTAL	Parts I–VII Online Theory Instruction	40.0	—

Sixteen of the 40 online hours — Part I subjects A through F plus the cumulative review — satisfy the federal and state pre-contact training requirement and must be completed and verified in TalentLMS before a student may check in for any skills laboratory session involving resident-care simulation.

Cross-reference: the 15 online modules delivered through TalentLMS map to the seven Parts and the 40-hour structure above. See the module-to-Part crosswalk and hour reconciliation in the GAHCTC Blended Course Map, which references the same modules as "Mod 01" through "Mod 15" in its chapter-by-chapter alignment table.

5.2 In-Person Skills Laboratory — 34 Hours (On-Site Check-In Required)

Students complete the online theory for a given curriculum part before checking in at the GAHCTC training facility for the corresponding skills laboratory session. Laboratory sessions emphasize instructor demonstration, student return demonstration, and hands-on competency check-off documented on the GAHCTC Skills Checklist, which is aligned with the Federal and State Core Curriculum Required Skills Checklist.

Part	Skills Laboratory Focus	Lab Hours
I	Hand hygiene (20-second handwash); donning and doffing PPE (gloves, gown, mask, eye protection); standard and transmission-based precautions; foreign body airway obstruction; body mechanics; gait belt application; safety and emergency procedures	8.0
II	Vital signs (temperature, pulse, respirations, blood pressure); height and weight (ambulatory, wheelchair, and bedridden methods); resident environment setup; recording and reporting	6.0
III	Bathing (complete and partial bed bath, perineal care); grooming and mouth care; dressing including hemiplegic technique; toileting; feeding technique; skin and pressure injury observation; positioning, turning, and transfers; occupied and unoccupied bedmaking	10.0
IV–V	Therapeutic communication with cognitively impaired residents; validation and redirection techniques; person-centered dementia care scenarios; behavioral de-escalation practice	4.0
VI	Ambulation with gait belt, cane, and walker; assistive device use; active and passive range of motion; bowel and bladder retraining documentation; prosthetic and orthotic device care; mechanical lift transfer (clinical preparation)	4.0
VII	Residents' rights scenarios (privacy, HIPAA, grievance process, ombudsman, restraint avoidance); cumulative skills review; comprehensive skills check-off; in-person final examination	2.0
TOTAL	In-Person Skills Laboratory at the GAHCTC Facility	34.0

All skills laboratory sessions take place at the GAHCTC training facility. Students must check in on arrival and complete the GAHCTC Skills Checklist with instructor initials and date for every required skill.

5.3 Clinical Orientation — 24 Hours (Affiliated Nursing Home)

Clinical orientation occurs at a licensed nursing home or long-term care facility holding a current signed affiliation agreement with GAHCTC. Students must have completed all 40 online hours and all 34 skills laboratory hours, including the comprehensive skills check-off, before clinical placement.

Clinical Day	Clinical Orientation Focus	Hours
Day 1	Facility orientation: tour, emergency codes and safety protocols, documentation systems (electronic and paper charting), shift reporting and handoff communication; Skills Validation Set 1 — vital signs, hand hygiene, PPE, residents' rights observation	8.0
Day 2	Supervised personal care: bathing, grooming, oral care, dressing, and toileting with an assigned resident; gait belt transfer and positioning under supervision; observation and reporting of abnormal findings; therapeutic communication and dignity at the bedside; clinical journal submission	8.0
Day 3	Final competency demonstrations: all applicable clinical skills validated (vital signs, transfers, personal care, restorative and range of motion activities, dementia care techniques); structured feedback session with the clinical supervisor; post-clinical program survey	8.0
TOTAL	Supervised Clinical Orientation at an Affiliated Nursing Home	24.0

5.4 Program Hour Verification Summary

Component	Hours	Running Total
Online Theory Instruction (Parts I–VII, self-paced TalentLMS)	40	40
In-Person Skills Laboratory (on-site check-in)	34	74
Clinical Orientation (affiliated nursing home)	24	98
GRAND TOTAL — exceeds the Georgia DCH 85-hour minimum	98	98

Completion Rule

All 98 approved program hours must be completed before a GAHCTC Certificate of Completion is issued and before the student is submitted as eligible for the Georgia Nurse Aide Competency Evaluation. Hours cannot be reduced, substituted, waived, or credited from prior experience, prior employment, or prior coursework.

SECTION 6: CURRICULUM CONTENT — SEVEN-PART FRAMEWORK

The GAHCTC blended NATP is organized into seven curriculum parts aligned with Nursing Assistant Care: The Basics, 6th Edition and the federal and state core curriculum required under 42 CFR §§ 483.150–483.158. Each part is introduced through online theory and reinforced through in-person skills laboratory practice and, where applicable, clinical orientation. All six subjects in Part I must be completed online and verified in TalentLMS before any skills laboratory check-in involving resident-care simulation or direct resident contact.

PART I — Pre-Contact Foundations of Nurse Aide Practice | Online: 16.0 hrs | Required BEFORE Skills Laboratory Check-In

Sub-Unit	Delivery	Content Topics	Textbook Ref.
I.A	Online	Communication and interpersonal skills, stress management, and chain of command: verbal and nonverbal communication; active listening; body language; chain of command and escalation; stress management and coping strategies; therapeutic communication; conflict de-escalation	Ch 2
I.B	Online	Infection control: chain of infection (six links and how to break each); standard precautions; CDC handwashing technique; PPE donning and doffing (gloves, gown, mask, eye protection); transmission-based precautions (contact, droplet, airborne); isolation room setup and maintenance; aseptic technique — clean versus sterile	Ch 2
I.C	Online	Safety, emergency procedures, foreign body airway obstruction, and body mechanics: safety hazards in long-term care; fall prevention and call light use; fire safety (RACE and PASS); emergency and evacuation procedures; FBAO in the conscious and unconscious adult and child; safe body mechanics for lifting and transfers	Ch 2
I.D	Online	Promoting residents' independence: restorative nursing principles; cueing, hand-over-hand, and setup-only approaches; supporting resident self-care in activities of daily living	Ch 1; Ch 3
I.E	Online	Respecting residents' rights (OBRA '87): privacy, dignity, self-determination and personal choice; freedom from abuse, neglect, and restraint; the right to make personal decisions; role and authority of the Long-Term Care Ombudsman; rights violations and mandatory reporting	Ch 1; Ch 3
I.F	Online	Legal and ethical behavior and scope of practice: the Georgia CNA scope; delegated versus non-delegated tasks; negligence, malpractice, and false imprisonment; ethical principles — beneficence, nonmaleficence, autonomy, justice; HIPAA and protected health information; penalties for violation; mandatory reporting obligations	Ch 2; Ch 3; Ch 5
—	Online	Cumulative review and assessment across Parts I.A through I.F. Passing this assessment is a prerequisite for skills laboratory check-in eligibility.	—

PART II — Assessment and Observation Skills | Online: 6.0 hrs | Skills Laboratory: 6.0 hrs

Sub-Unit	Delivery	Content Topics	Textbook Ref.
II.A	Online + Lab	Vital signs: temperature (oral, rectal, axillary, tympanic); pulse (rate, rhythm, character); respirations (rate and character); blood pressure technique and equipment; normal ranges and reporting parameters	Ch 7
II.B	Online + Lab	Height and weight: measurement techniques for ambulatory, wheelchair, and bedridden residents; clinical significance of change	Ch 7

Sub-Unit	Delivery	Content Topics	Textbook Ref.
II.C	Online + Lab	Resident environment: cleanliness, organization, and safety of the resident's living space	Ch 7
II.D	Online	Recognizing abnormal changes (report all to the supervising nurse): cardiorespiratory signs; gastrointestinal and urinary signs; behavioral and functional change; nutrition, hydration, and weight change; skin change	Ch 4; Ch 6
II.E	Online	Pain recognition: numeric and observational pain scales; verbal and nonverbal pain cues; accurate reporting to the supervising nurse	Ch 6
II.F	Online	End-of-life and postmortem care: signs of approaching death; comfort measures; cultural and spiritual considerations; postmortem care procedure	Ch 3

PART III — Personal Care and Clinical Skills | Online: 6.0 hrs | Skills Laboratory: 10.0 hrs

Sub-Unit	Delivery	Content Topics	Textbook Ref.
III.A	Online + Lab	Bathing: complete bed bath; partial bath; perineal care; pressure injury observation during bathing	Ch 6
III.B	Online + Lab	Grooming and mouth care: oral hygiene; denture care; hair, nail, and shave care	Ch 6
III.C	Online + Lab	Dressing: adaptive techniques; hemiplegic dressing sequence — affected side first when dressing, unaffected side first when undressing	Ch 6
III.D	Online + Lab	Toileting: bedpan; bedside commode; bathroom assistance; infection control throughout all toileting procedures	Ch 6
III.E	Online + Lab	Eating and hydration assistance: aspiration risk identification; therapeutic positioning during meals; adaptive utensils; dignity in meal assistance; intake documentation and refusal reporting	Ch 8
III.F	Online + Lab	Feeding technique: step-by-step procedure for the dependent resident; positioning, pace, bite size, and communication during feeding	Ch 8
III.G	Online + Lab	Skin care and pressure injuries: Braden scale and risk factors; pressure injury staging; skin tears; preventive skin care during daily care	Ch 6
III.H	Online + Lab	Transfers, positioning, and turning: gait belt application and transfer technique; mechanical lift operation and safety; turning and positioning in bed and chair; correct body alignment; pressure injury observation during repositioning	Ch 6
III.I	Lab	Bedmaking: occupied and unoccupied bed, including linen handling and infection control	Ch 6

PART IV — Mental Health and Social Service Needs | Online: 3.0 hrs | Reinforced in Lab Parts IV–V: 4.0 hrs

Sub-Unit	Delivery	Content Topics	Textbook Ref.
IV.A	Online	Modifying aide behavior in response to depression: identifying signs of depression; adapting communication and response	Ch 2; Ch 5
IV.B	Online	Developmental tasks of aging: Erikson's developmental stages; normal physical and psychological changes of aging	Ch 3
IV.C	Online	Responding to resident behavior: person-centered response strategies; supporting dignity, choice, and autonomy	Ch 3

Sub-Unit	Delivery	Content Topics	Textbook Ref.
IV.D	Online	Personal choice and dignity: allowing and reinforcing behavior consistent with resident dignity and preference	Ch 3
IV.E	Online	Family as emotional support: engaging family as part of the care team; supporting family involvement	Ch 3
IV.F	Online	Emotional, mental health, spiritual, and cultural needs: recognizing and supporting emotional and mental health needs; respecting spiritual and cultural identity in care	Ch 3; Ch 5

PART V — Care of the Cognitively Impaired Resident | Online: 3.0 hrs | Reinforced in Lab Parts IV–V: 4.0 hrs

Sub-Unit	Delivery	Content Topics	Textbook Ref.
V.A	Online	Techniques for dementia and Alzheimer's care: unique needs and behaviors; the person-centered dementia care approach	Ch 2; Ch 5
V.B	Online + Lab	Communicating with cognitively impaired residents: therapeutic communication strategies; validation and redirection techniques	Ch 2; Ch 5
V.C	Online	Understanding behavior: behavioral and psychological symptoms of dementia — stages and expressions	Ch 5
V.D	Online + Lab	Appropriate responses to behavior: non-pharmacological behavioral interventions; de-escalation strategies	Ch 5
V.E	Online	Reducing the effects of cognitive impairment: environmental modification; consistent routines; structured activity	Ch 5
V.F	Online	Spiritual and cultural health in dementia care: maintaining spiritual and cultural identity throughout cognitive decline	Ch 3

PART VI — Basic Restorative Services | Online: 3.0 hrs | Skills Laboratory: 4.0 hrs

Sub-Unit	Delivery	Content Topics	Textbook Ref.
VI.A	Online	Resident self-care: goal setting within the restorative care plan; the nurse aide's role in restorative nursing	Ch 3
VI.B	Online + Lab	Ambulation and gait belt: correct gait belt application; ambulation with and without assistive devices	Ch 9
VI.C	Online + Lab	Assistive devices: walkers, canes, wheelchairs, and adaptive equipment — safe use, storage, and maintenance	Ch 6; Ch 8; Ch 9
VI.D	Online + Lab	Range of motion: active, passive, and active-assistive range of motion in a full-body sequence; therapeutic positioning in bed and chair	Ch 6; Ch 9
VI.E	Online	Bowel and bladder retraining: scheduled toileting programs; output documentation and the bladder and bowel log	Ch 9
VI.F	Online + Lab	Prosthetic and orthotic devices and eyeglasses: application, removal, care, and storage; safety and skin integrity checks	Ch 6; Ch 8

PART VII — Residents' Rights | Online: 3.0 hrs (incl. cumulative assessment) | Skills Laboratory: 2.0 hrs (incl. in-person final)

Sub-Unit	Delivery	Content Topics	Textbook Ref.
VII.A	Online	Privacy and confidentiality: HIPAA overview; protected health information — safeguards and documentation	Ch 1; Ch 7
VII.B	Online	Personal choice and autonomy: preference versus safety-based restriction; honoring resident decisions	Ch 1
VII.C	Online	Grievances and the Ombudsman: the facility grievance process; Ombudsman role, authority, and how residents access services	Ch 1; Ch 3
VII.D	Online	Resident and family groups and activities: encouraging and supporting participation in facility activities and family councils	Ch 3; Ch 7
VII.E	Online	Security of personal possessions: property inventory documentation; reporting missing items	Ch 1
VII.F	Online	Freedom from abuse: physical, verbal, mental, sexual, and financial abuse; neglect and misappropriation; mandatory reporting procedure and obligations	Ch 1; Ch 7
VII.G	Online + Lab	Restraint avoidance: restraint alternatives; least-restrictive approaches; legal restrictions on restraint use. Cumulative online assessment and in-person comprehensive final examination (Parts I–VII).	Ch 1

SECTION 7: TEACHING FOUNDATION, ADULT LEARNING & FACULTY

7.1 Components of a Teaching Foundation

Component	Description
Subject Matter Mastery	Current, evidence-based command of all content in the Hartman Basics 6th Edition curriculum, delivered effectively through online modules and in-person demonstration.
Pedagogical and Digital Knowledge	Skill in teaching diverse adult learners using TalentLMS tools — recorded instruction, knowledge checks, quizzes, chapter tests — alongside hands-on skills instruction.
Curriculum Alignment	Online module objectives, laboratory instruction, and assessments are systematically aligned to OBRA '87 and Georgia DCH standards.
Learning Environment	Physical and psychological safety, inclusivity, and respect are maintained in the online environment, the skills laboratory, and the clinical site.
Assessment Literacy	Valid, reliable methods are used to evaluate and document learning across the cognitive, affective, and psychomotor domains, using LMS analytics and instructor-verified skills checklists.
Professional Modeling	Instructors model the professional behavior and ethical conduct expected of nurse aides, including appropriate online communication etiquette.

7.2 Characteristics of Adult Learners in a Blended Setting

- Self-concept — adult learners move toward increasing self-direction and autonomy, supported by self-paced online modules with flexible access.
- Experience — adults bring a reservoir of life and work experience as a resource for new learning, shared through online discussion and in-person group activity.
- Readiness to learn — learning is oriented toward the developmental tasks of social and professional roles.
- Orientation to learning — adults seek immediate, practical application; online theory is reinforced through in-person hands-on practice.
- Motivation to learn — adults are internally motivated by personal and professional growth; the blended format accommodates work and family schedules.

7.3 Factors that Influence Adult Learning in a Blended Format

- Prior life and work experience — adults connect new online learning to existing knowledge.
- Intrinsic motivation — adults learn best when content is relevant to their goals and accessible on a flexible schedule.
- Problem-centered learning — adults prefer real-world scenarios such as online case studies, video demonstration, and in-person role play.
- Cultural and linguistic diversity — instructors use culturally responsive strategies in both online content and in-person sessions.
- Digital readiness — comfort with the learning platform, device access, and internet reliability directly affect online progress and must be assessed early.
- Learning barriers — time constraints, family responsibility, reading level, and technology access are identified at orientation and addressed proactively.

7.4 Role of the Instructor in Creating a Positive Learning Climate

- Establish trust, mutual respect, and psychological safety from the first online module and the first on-site check-in.
- Communicate clear expectations, module objectives, evaluation criteria, and skills laboratory check-in procedures.
- Use varied instructional strategies including recorded instruction, video demonstration, case studies, in-person role play, return demonstration, group activity, and guest speakers.
- Provide timely, constructive, and specific feedback on online assessment performance and in-person skills demonstration.
- Monitor TalentLMS progress reports at least weekly and intervene early with students falling behind the cohort pacing guide.
- Create opportunities for active participation, skill practice, and reflective learning through both online assignments and hands-on laboratory sessions.
- Assign meaningful written reflection (for example, "My Goals in Healthcare" and "A Stressful Situation") submitted through TalentLMS.
- Uphold professional, ethical, and culturally competent standards of conduct at all times, including in online communication.

7.5 Instructor Qualifications and Responsibilities

Role	Qualifications and Responsibilities
Program Director / Primary Instructor	Registered Nurse licensed in Georgia with a minimum of two (2) years of nursing experience, including at least one (1) year in long-term care. Responsible for curriculum integrity, hour verification, student records, DCH reporting, skills check-off oversight, and final grade determination.
Skills Laboratory Instructor	Registered Nurse meeting Georgia instructor qualification requirements, or a qualified supplemental instructor teaching under the direct supervision of the RN Program Director within the limits permitted by Georgia DCH.
Clinical Supervisor / Preceptor	Registered Nurse meeting Georgia qualification requirements, present on site and supervising students at all times during clinical orientation, at a ratio not exceeding 12:1.
All Instructional Staff	Current licensure verification, criminal background clearance, tuberculosis screening, current BLS certification, orientation to this manual and the GAHCTC Instructor Handbook, and annual faculty development documented in the personnel file.

7.6 Faculty Development Objectives

The following objectives apply to GAHCTC instructional staff, not to students. They are used in instructor onboarding and annual faculty development, and are documented in each instructor's personnel file. In prior versions of this manual these objectives appeared under program learning objectives, where they were misplaced.

1. State the OBRA '87 and Georgia DCH requirements for approval of a nurse aide training program, including 42 CFR §§ 483.150–483.158.
2. Summarize the components of a strong teaching foundation in nurse aide education, including in a blended delivery environment.
3. Identify the factors that influence adult learning online, including motivation, life experience, self-direction, and readiness to learn.
4. Apply andragogical principles to instructional planning for both online modules and in-person laboratory sessions.

5. Describe the instructor's role in promoting a positive, inclusive, and safe learning climate online and in person.
6. Identify the stages of the ADDIE instructional design model as applied to blended curriculum development.
7. Explain the purpose of each stage of instructional design and its application to nurse aide education.
8. Describe the principles for developing measurable behavioral objectives across the cognitive, affective, and psychomotor domains.
9. Develop two behavioral objectives in each domain appropriate to online and in-person delivery.
10. Construct multiple-choice items aligned with cognitive-level objectives and consistent with the NNAAP® written examination format.
11. Illustrate one complete lesson from a designated unit of study, incorporating objectives, content, methods, evaluation, and test items appropriate to blended delivery.

SECTION 8: INSTRUCTIONAL DESIGN POLICY

8.1 The ADDIE Instructional Design Model

GAHCTC uses the ADDIE model to develop, deliver, and evaluate all blended program curriculum. Each module follows this framework.

Stage	Purpose and Application (Blended Track)
Analysis	Identify learner needs, Georgia DCH and OBRA requirements, knowledge gaps, and available resources — Hartman Basics 6th Edition, TalentLMS, the GAHCTC skills laboratory, and affiliated clinical sites.
Design	Develop module learning objectives across the cognitive, affective, and psychomotor domains; design online assessment strategy (knowledge checks, module quizzes, chapter tests) and in-person instructional methods aligned to core curriculum requirements.
Development	Build online modules (recorded instruction, readings, video), in-person lesson plans, skills checklists, role-play scenarios, case studies, reflection assignments, and item banks in TalentLMS.
Implementation	Deliver instruction across the TalentLMS campus, the in-person skills laboratory (demonstration, return demonstration, check-off), and clinical orientation.
Evaluation	Assess learning through module quizzes, chapter tests, the in-person final examination, skills check-offs, preceptor observation, and cohort outcome data; feed results into Section 16 quality improvement.

8.2 Module and Session Structure

Every online module and every in-person skills laboratory session must include the following five components, consistent with the GAHCTC blended lesson plan template.

- Objectives — measurable behavioral objectives appropriate to online or in-person delivery.
- Content — organized sub-units with specific Hartman 6th Edition chapter references, delivered through recorded online instruction or in-person demonstration.
- Methods and materials — instructional strategies and required resources, including LMS media, discussion prompts, handouts, equipment, and skills checklists.
- Evaluation method — the specific assessment strategy for that module or session: knowledge check, module quiz, chapter test, return demonstration, written reflection, or clinical check-off.
- Test items — a minimum of three items aligned with the session's cognitive-level objectives, delivered through TalentLMS or as part of the in-person final examination.

SECTION 9: BEHAVIORAL OBJECTIVES POLICY

9.1 Principles for Writing Behavioral Objectives

All learning objectives used in the GAHCTC NATP must be measurable and must contain the following four components.

- Audience — who will perform the behavior (for example, "The student will...").
- Behavior — the observable, measurable action, expressed with a Bloom's taxonomy action verb.
- Condition — the circumstances under which the behavior occurs (for example, "after completing the Part I.B online module," "during the in-person skills laboratory").
- Degree — the standard of performance, stated in terms consistent with the GAHCTC grading standards in Section 10.3: a minimum of 80% on written assessment, or 100% with no critical error on skills performance.

Objectives written before this revision that cite a passing standard of 75% or 85% are superseded. All written assessment objectives use 80%; all psychomotor objectives use 100% with no critical error.

9.2 Cognitive Domain Objectives — Examples

Example 1. After completing the Part VII online module, the student will correctly identify at least three residents' rights under OBRA '87 that were violated in a written case scenario, scoring a minimum of 80% on the module quiz.

Example 2. Without reference materials, the student will list the six links in the chain of infection and describe at least one method of breaking each link, scoring a minimum of 80% on the Part I.B module quiz.

9.3 Affective Domain Objectives — Examples

Example 1. During clinical orientation, the student will demonstrate respect for resident dignity and cultural preference in 100% of documented care interactions, as observed and evaluated by the clinical preceptor using the GAHCTC Clinical Competency Checklist.

Example 2. Upon completing the Part V dementia care module, the student will submit a written reflection through TalentLMS demonstrating cultural humility in resident interaction, incorporating at least two therapeutic communication strategies and one personal or professional experience, and meeting a minimum of 80% of the reflection rubric criteria.

9.4 Psychomotor Domain Objectives — Examples

Example 1. Using a mannequin during the Part I.B skills laboratory session, the student will perform handwashing technique, completing 100% of the steps on the GAHCTC Hand Hygiene Skills Checklist with no critical error, as verified by the RN instructor during return demonstration.

Example 2. During clinical competency check-off, the student will independently perform a complete bed bath including perineal care on an assigned resident, adhering to infection control standards and maintaining privacy and dignity throughout, completing 100% of the checklist steps with no critical error.

SECTION 10: ASSESSMENT, EVALUATION & PROGRAM COMPLETION

10.1 Assessment Philosophy

GAHCTC uses a competency-based assessment model that evaluates knowledge acquisition online and skill demonstration in person across all seven curriculum parts. Assessments are aligned to module learning objectives, Hartman Basics 6th Edition content, and the NNAAP® competency evaluation format used for the Georgia CNA examination administered by Credentia.

10.2 Assessment Architecture

The blended track uses a four-tier assessment structure. Tiers one through three are delivered online through TalentLMS; the final examination is administered in person.

Tier	Assessment	Delivery	Purpose and Rules
1	Knowledge check	Online — TalentLMS	Ungraded formative check embedded within each module. Unlimited attempts. Does not affect the final grade but must be attempted before the module is marked complete.
2	Module quiz	Online — TalentLMS	One graded quiz for every module. Minimum passing score 80%. One retake permitted. The module does not unlock the next module until the quiz is passed.
3	Chapter test	Online — TalentLMS	One graded test for every textbook chapter. Minimum passing score 80%. One retake permitted following documented remediation with the instructor.
4	Comprehensive final examination	In person — GAHCTC facility	Administered on site during the Part VII skills laboratory session, together with the comprehensive skills check-off. Minimum passing score 80%. Not available online under any circumstance.

Why the Final Is In Person

The comprehensive final examination is proctored on site to protect examination integrity and to mirror the conditions of the NNAAP® written examination. Remote proctoring, take-home administration, and open-book administration are not permitted. A student who cannot attend the scheduled final examination date must request rescheduling in writing from the Program Director in advance; unexcused absence from the final results in an incomplete and deferral to the next cohort.

10.3 Grading and Progression Standards

Assessment Type	Minimum Passing Standard	Retake Opportunity
Module quizzes and chapter tests (TalentLMS)	80%	One retake per assessment
Comprehensive final examination (in person)	80%	One retake, scheduled at the Program Director's discretion
Skills return demonstration and check-off (laboratory)	100% of checklist steps, no critical error	One retake per skill

Assessment Type	Minimum Passing Standard	Retake Opportunity
Clinical competency check-off (clinical orientation)	100% of checklist steps, no critical error	With clinical supervisor approval
Online module completion	100% — all 40 online hours	Make-up only with Program Director approval
Skills laboratory attendance	100% — all 34 laboratory hours	Make-up only with Program Director approval
Clinical orientation attendance	100% — all 24 clinical hours	Make-up only with Program Director approval

Standardization notice: prior versions of this manual cited 75% in one sample module and 85% in two sample objectives. The uniform GAHCTC standard is 80% for all written assessment and 100% with no critical error for all skills performance. All lesson plans, item banks, rubrics, and TalentLMS assessment settings must be reconciled to this standard.

10.4 Remediation

- A student who does not meet the 80% standard on a module quiz or chapter test must complete documented remediation with the instructor before the single permitted retake.
- Remediation may include re-reading assigned content, a review session, additional practice items, or a written learning plan, and is documented in the student record.
- A student who does not pass a retake is placed on academic warning and meets with the Program Director to determine whether continuation in the current cohort is feasible.
- A skills check-off that is not passed on the retake requires additional supervised laboratory practice before a further attempt is scheduled; a student who cannot demonstrate a required skill safely may not proceed to clinical orientation.

10.5 Program Completion and Certificate of Completion

A GAHCTC Certificate of Completion is issued only when every one of the following conditions is satisfied. Partial completion does not qualify a student for the certificate or for submission as eligible for the state competency evaluation.

- All 98 approved program hours completed and documented — 40 online, 34 skills laboratory, 24 clinical orientation.
- All module quizzes and chapter tests passed at 80% or above.
- Comprehensive in-person final examination passed at 80% or above.
- Every skill on the GAHCTC Skills Checklist verified at 100% with no critical error and initialed and dated by the RN instructor.
- Clinical competency check-off completed satisfactorily and signed by the clinical supervisor.
- All tuition and fees paid in full.
- All required documentation on file, including background clearance, tuberculosis screening, and signed agreements.

SECTION 11: SAMPLE BLENDED MODULE & SKILLS LABORATORY PLAN

The following illustrates a complete module using the GAHCTC blended lesson plan format. All online modules and corresponding skills laboratory sessions follow this structure and are maintained in the Program Director's curriculum file.

Element	Detail
Unit / Part	Part I.B — Infection Control
Delivery Format	Self-paced online module in TalentLMS (recorded instruction, knowledge check, module quiz), followed by in-person skills laboratory check-in
Textbook	Nursing Assistant Care: The Basics, 6th Edition — Chapter 2
Duration	Online: 2.5 hours In person: portion of the 8.0-hour Part I laboratory session
Regulatory Basis	OBRA '87; Georgia DCH nurse aide training requirements; CDC standard precautions
Prerequisite	Module I.A completed; this module is part of the 16-hour pre-contact requirement

Module Learning Objectives — The student will:

1. (Cognitive) Describe the chain of infection and identify how to break at least two links using standard precautions quiz, 80% minimum.
2. (Psychomotor) Demonstrate correct handwashing technique, completing 100% of the steps on the GAHCTC Skills Checklist with no critical error — in-person return demonstration.
3. (Cognitive) Identify standard and transmission-based precautions and describe the role of PPE in infection prevention — module quiz, 80% minimum.
4. (Psychomotor) Demonstrate correct donning and doffing of PPE — gloves, gown, mask, and eye protection — per CDC protocol, with no critical error, during in-person return demonstration.
5. (Application) Analyze an online case study of infection control breaches and correctly identify each error, scoring 80% or above.

Methods and Materials

Online: recorded instructor presentation with a TalentLMS discussion prompt; embedded video demonstration of handwashing technique; case study identifying infection control breaches; downloadable standard precautions quick-reference handout. In person: instructor demonstration at the skills laboratory sink; student return demonstration of handwashing and PPE donning and doffing; small-group chain of infection review.

Evaluation Methods

TalentLMS module quiz for Part I.B; in-person return demonstration check-off for handwashing and PPE recorded on the GAHCTC Skills Checklist; instructor observation throughout the laboratory session. Content from this module also appears on the Chapter 2 test and on the comprehensive in-person final examination.

Sample Test Items

- List the six links in the chain of infection.
- Identify the three categories of transmission-based precautions and give one example of each.
- True or false, with rationale: wearing gloves eliminates the need for handwashing.

SECTION 12: STUDENT POLICIES & CONDUCT

GA Healthcare Training & Consulting maintains a professional, respectful, and productive learning environment across the online campus, the skills laboratory, and the clinical setting. All participants adhere to the following policies as a condition of enrollment. Failure to comply may result in disciplinary action up to and including dismissal from the program.

12.1 Attendance and Completion Policy

Attendance and module completion are mandatory for online instruction, skills laboratory sessions, and clinical orientation. Because program hours are set by Georgia DCH and federal regulation, every component is essential to program completion and to eligibility for the Georgia Nurse Aide Competency Evaluation.

- Participants must complete all 40 online hours, including all module content, assignments, quizzes, and chapter tests, with completion timestamps recorded in TalentLMS.
- Although online instruction is self-paced, each cohort follows a published pacing guide. A student more than one module behind the pacing guide will be contacted by the instructor and placed on a written catch-up plan.
- Participants must check in on site and be present for the full duration of every scheduled skills laboratory session — 34 hours total.
- Tardiness of more than 15 minutes to a skills laboratory session or a clinical orientation day constitutes a half-day absence.
- Each participant is allowed one make-up opportunity for either a missed skills laboratory session or a missed clinical orientation day — not both.
- Missing more than one in-person session, for any reason, requires the participant to defer to the next available cohort. Program hour requirements cannot be satisfied with more than one absence.
- Make-up sessions must be scheduled with and approved by the Program Director, in advance of the next scheduled in-person session whenever possible.
- Written reflection assignments must be submitted through TalentLMS by the assigned deadline. Late submission requires Program Director approval.

Participants withdrawn for incomplete online modules or excessive in-person absence who wish to re-enroll must apply for the next available cohort and will be subject to the tuition rate in effect at the time of re-enrollment.

12.2 Tuition, Fees, and Refund Policy

- Tuition is due in full before TalentLMS credentials are released and before the program start date. Enrollment is not confirmed until payment is received or a written payment plan is approved.
- A payment plan may be considered case by case at the discretion of the Program Director and CEO. Requests must be submitted in writing before the program start date.
- Participants on an approved payment plan must remain current. Failure to maintain the schedule may result in suspension of TalentLMS access until the balance is resolved.
- A \$25.00 student professional liability insurance fee is required of every student and is billed separately from tuition. Coverage must be in force before the first clinical day (Section 13.7).
- All fees — including supply, materials, background check, uniform, liability insurance, and registration fees — must be paid in full before a participant may check in for skills laboratory sessions or be submitted as eligible for the competency evaluation.

- Refunds are governed by the GAHCTC Refund Policy and the itemized cost schedule provided at enrollment, which are incorporated into this manual by reference and provided to every student before the enrollment agreement is signed.
- The state competency evaluation fee is paid separately to Credentia and is not included in GAHCTC tuition. Under federal and Georgia law, a nursing facility that employs a nurse aide may be required to pay that fee on the employee's behalf.

12.3 Code of Conduct

Respect and Professionalism

- Treat all instructors, peers, residents, facility staff, and guests with dignity, courtesy, and respect at all times, including in online discussion, messages, and virtual sessions.
- Profanity and offensive language are prohibited in online communication, the skills laboratory, and the clinical area, and will result in disciplinary action up to and including dismissal.
- Disruptive behavior — including inappropriate online posts, argumentative conduct in discussion threads, or disrespect toward instructors or peers in person — will not be tolerated.
- Participants are expected to engage actively in online modules and in-person sessions and to support a positive, inclusive environment for all classmates.
- Report suspected abuse, neglect, or mistreatment of a resident immediately, in accordance with Georgia mandated reporter law.

Technology and Online Learning Platform

- Participants are responsible for maintaining reliable internet access and a compatible device sufficient to complete all online modules, quizzes, and chapter tests.
- TalentLMS login credentials are confidential and must not be shared. Sharing credentials or allowing another person to complete coursework is an academic integrity violation.
- Technical issues must be reported promptly to the Program Director or designated support contact. Extensions for documented technical failure are at the discretion of the Program Director.
- Recording, downloading, redistributing, or reposting GAHCTC instructional content is prohibited. Course materials are the intellectual property of GAHCTC and its licensors.

Cell Phones and Electronic Devices — In-Person Sessions

- Talking and texting on personal devices is not permitted during any skills laboratory session. Phones must be silenced and stored out of sight during instructional time.
- Cell phones and personal electronic devices are not permitted at the clinical site at any time. Devices must be turned off and secured off the unit. See Section 13.5.
- In a personal or family emergency, the participant must notify the clinical supervisor and step outside the clinical area before using a phone, then return promptly.
- Repeated violations result in formal disciplinary action and may jeopardize standing in the program.
- Smartwatches and wearable devices must be silenced and must not be used for communication during in-person instruction or clinical hours.

Confidentiality and HIPAA

- Strict confidentiality of all patient and resident information must be maintained at all times, including in online discussion posts and reflection assignments. No patient-identifiable information may ever be shared online.

- Photographs, audio recordings, and video recordings of residents, clinical environments, or any patient-identifiable information are strictly prohibited.
- Violation of resident confidentiality results in immediate dismissal from the program and will be reported to the Georgia Nurse Aide Registry and the clinical facility.

Academic Integrity

- Cheating online quizzes or tests, plagiarism in written submissions, falsification of clinical records or attendance, and unauthorized assistance during skills evaluation are strictly prohibited.
- Because online assessments are unproctored, GAHCTC monitors TalentLMS analytics — including time on task, attempt patterns, and answer patterns — for indicators of integrity violation.
- Participants found in violation are subject to immediate dismissal and may be reported to the Georgia Nurse Aide Registry.

12.4 Dress Code and Appearance — In-Person Sessions

Professional appearance is required for all skills laboratory and clinical orientation sessions. Participants who arrive out of uniform may be sent home to change; time missed is recorded as tardy or absence depending on the time elapsed. There is no dress code for online instruction.

Uniform

- Skills laboratory check-in: navy blue scrubs with the GAHCTC program logo.
- Clinical orientation: GAHCTC navy blue uniform scrubs with GAHCTC or facility-issued identification badge visible always.
- All scrubs must be clean, pressed, and in good condition. Torn, faded, or soiled scrubs are not acceptable.

Footwear

- Closed-toe, closed-heel, non-slip professional shoes or nursing clogs only.
- No sandals, flip-flops, or open-toe or open-heel footwear of any kind.
- Shoes must be clean and in good condition.

Hair, Jewelry, and Nails

- Hair must be clean, neat, and secured away from the face and off the shoulders during all in-person sessions. Loose hair is a safety and infection control hazard.
- Jewelry must be minimal and non-dangling. Rings with stones are not permitted during clinical activity.
- Nails must be natural, clean, and trimmed short. Acrylic, gel, and decorative nails are not permitted during clinical orientation because they pose an infection control risk.
- Heavy fragrance should be avoided; residents may have sensitivities or allergies.

Identification

- The GAHCTC identification badge must be always worn and visible in the skills laboratory and clinical setting.
- Participants arriving without an identification badge may be denied entry and marked absent for that session.

12.5 Student Records and Privacy

- GAHCTC maintains a record for every student containing admission documentation, background and health clearance, attendance and hour verification, TalentLMS completion and score reports, skills checklists, clinical evaluations, disciplinary records, and grievance records.
- Student records are retained for a minimum of three (3) years and stored securely with access limited to the Program Director, CEO, and personnel with a legitimate instructional need.
- Students may inspect their own record on written request and may request correction of a factual error through the Program Director.
- GAHCTC does not release student records to a third party without written authorization from the student, except as required by Georgia DCH, CMS, a clinical affiliate under the terms of its agreement, or applicable law.

12.6 Health, Safety, and Incident Reporting

- Any injury, needlestick, exposure to blood or body fluid, resident incident, or near miss occurring during a skills laboratory session or clinical orientation must be reported immediately to the supervising instructor or clinical supervisor and documented on a GAHCTC incident report the same day. The step-by-step clinical incident procedure is set out in Section 13.6.
- Students follow the host facility's exposure control plan during clinical orientation and are oriented to it on Clinical Day 1.
- GAHCTC does not provide health insurance or workers' compensation coverage for students. Students are responsible for the cost of any medical evaluation or treatment arising from an injury or exposure.
- A student who becomes ill or is injured such that they cannot safely perform required activities must provide written clearance from a licensed healthcare provider before returning to laboratory or clinical activity.
- Students must not report to a skills laboratory session or clinical site while impaired. GAHCTC maintains a drug-free and alcohol-free learning environment; clinical facilities may impose additional screening requirements.

SECTION 13: CLINICAL ORIENTATION POLICY

13.1 Clinical Requirements

Georgia-approved nurse aide training programs must be a minimum of 85 hours in length, inclusive of theory, laboratory, and supervised clinical practice. The GAHCTC blended NATP provides 98 total program hours, of which a minimum of 24 are supervised clinical orientation hours completed at a licensed nursing home. All Part I online pre-contact training and the full 34-hour skills laboratory, including comprehensive skills check-off, must be completed before a student participates in direct resident care.

- Total clinical orientation: 24 hours delivered over three 8-hour clinical days.
- All clinical hours occur under the direct supervision of a qualified RN clinical supervisor or preceptor.
- Students must demonstrate laboratory competency in all foundational skills, verified through skills check-off, before clinical placement.
- Student-to-supervisor ratio does not exceed 12:1 at any clinical site.
- Every clinical site holds a current signed affiliation agreement with GAHCTC.

13.2 Clinical Orientation Progression

- Clinical Day 1 — Report at 7:00 a.m. (arrive 6:45 a.m. for preconference); facility tour; emergency codes and safety protocols; documentation systems; shift reporting and handoff communication; Skills Validation Set 1 (vital signs, hand hygiene, PPE, residents' rights observation); orientation to the facility exposure control plan.
- Clinical Day 2 — Supervised personal care including bathing, grooming, oral care, dressing, and toileting; gait belt transfer and positioning under supervision; observation and reporting of abnormal findings; therapeutic communication and dignity at the bedside; clinical journal submission.
- Clinical Day 3 — Final competency demonstrations across all applicable clinical skills; structured feedback session with the clinical supervisor; post-clinical program survey.

13.3 Pre-Clinical Requirements

Before clinical placement, students must have on file:

- Completed criminal background check with results satisfactory under Georgia DCH standards.
- Current immunization and screening documentation per facility requirement, including tuberculosis screening.
- Current CPR/BLS certification at the healthcare provider level.
- Signed HIPAA confidentiality agreement.
- Signed clinical practicum agreement.
- Health clearance from a licensed healthcare provider dated within twelve months.
- Proof of student professional liability insurance in force (\$25.00 fee — see Section 13.7).
- Signed acknowledgment of the clinical site conduct standards in Section 13.5, including the prohibition on cell phones and on photographing residents.
- Verified completion of all 40 online hours and all 34 skills laboratory hours, including comprehensive skills check-off.

13.4 Clinical Schedule, Punctuality, and Preconference

Clinical orientation days run on the facility's schedule, not the program's. Students are expected on the unit and ready to receive assignment at the start of the shift.

Time	Expectation
6:45 a.m.	Recommended arrival. Students are strongly encouraged to arrive by 6:45 a.m. for preconference with the clinical instructor — assignment review, objectives for the day, questions from the previous day, and any facility announcements.
7:00 a.m.	Clinical begins. Students must be on the unit, in full uniform with identification badge visible, signed in, and ready to receive assignment. This is a hard start time.
During the shift	Students remain with the assigned resident group and the supervising instructor or preceptor. Breaks are taken only as directed by the clinical supervisor.
End of shift	Post-conference debriefing with the clinical instructor before departure. Students do not leave the site without checking out with the instructor.

- A student who arrives after 7:00 a.m. is tardy. Tardiness of more than 15 minutes constitutes a half-day absence under Section 12.1.
- A student who will be late or absent must notify the clinical instructor by telephone before 6:45 a.m. Text messages and messages left with classmates are not acceptable notice.
- Preconference is where the day's learning objectives, resident assignments, and safety concerns are set. A student who misses preconference may be assigned to observation only for that shift at the instructor's discretion.

13.5 Conduct at the Clinical Site

Students are guests of the clinical facility. Conduct at the clinical site reflects on the student, on GAHCTC, and on the program's ability to maintain its affiliation agreements. The following standards are non-negotiable.

Facility Rules and Chain of Command

- Students comply with all facility rules, regulations, policies, and procedures, including any that are stricter than GAHCTC policy. Where the two differ, the stricter standard applies.
- Students follow the direction of the GAHCTC clinical instructor and the facility charge nurse, and use the chain of command to raise any question, concern, or change in resident condition.
- Students perform only tasks within the Georgia nurse aide scope of practice on which they have been checked off, and only under supervision.
- Students never accept a resident care assignment independently, never accept delegation from anyone other than the assigned supervisor, and never perform a task they have not been taught.
- Students treat facility staff, residents, families, and visitors with courtesy and respect at all times, including when a request is refused or a correction is given.

Active Engagement — Expected

Clinical orientation is participatory. Students who stand back, cluster with classmates, or wait to be told what to do are not meeting the expectation and will be counseled; continued disengagement may result in removal from the rotation.

- Seek out care opportunities. Ask the instructor or preceptor for the next task rather than waiting to be assigned.
- Answer call lights promptly and within scope, and report to the nurse anything outside scope.
- Ask questions. Report observations. Volunteer for skills you need practice on.

- Remain with your assigned resident group and on your assigned unit. Do not congregate at the nurses' station, in break rooms, or in hallways.
- Complete the clinical journal and any assigned documentation the same day.

Cell Phones and Electronic Devices — Prohibited at the Clinical Site

- Cell phones and all personal electronic devices are NOT permitted at the clinical site at any time. Phones must be turned off and left in a locker, in a vehicle, or wherever the facility directs.
- Smartwatches and other wearable devices must be removed or fully silenced and must not be used for any form of communication during clinical hours.
- In a personal or family emergency, the student notifies the clinical instructor, steps entirely outside the clinical area, and returns promptly.
- A phone seen in use in a resident care area will be documented as a policy violation. A repeat violation results in removal from the clinical rotation.

HIPAA, Confidentiality, and Photography

- All resident information is confidential under HIPAA. What is seen, heard, or read at the clinical site stays at the clinical site.
- DO NOT take photographs, video, or audio recordings of residents, staff, families, charts, computer screens, name boards, or any part of the clinical environment. There is no exception to this rule, including for a student's own study use.
- Do not post about clinical experiences on social media. Do not discuss residents in hallways, elevators, break rooms, parking lots, or with family and friends.
- No resident-identifiable information — name, room number, diagnosis, photograph, or any detail that could identify a person — may appear in a clinical journal, reflection assignment, or TalentLMS post.
- Do not access a resident record you have not been assigned to, and do not remove any document, label, or printout from the facility.

Zero Tolerance

A HIPAA violation, or photographing or recording a resident or the clinical environment, results in immediate removal from the clinical site and dismissal from the program. The incident will be reported to the clinical facility and to the Georgia Nurse Aide Registry, and may carry civil and criminal penalties under federal law independent of any action taken by GAHCTC.

13.6 Incidents at the Clinical Site

An incident is any unplanned event involving a resident, a student, a staff member, or the environment — including a fall or near fall, a medication or care error, a needlestick or exposure to blood or body fluid, a student injury, a resident complaint, an equipment failure, a breach of confidentiality, or a conflict with staff. Incidents are learning opportunities, and reporting one promptly is expected professional behavior, not an admission of fault.

1. Ensure immediate safety. Stay with the resident and call for the nurse.
2. Notify the GAHCTC clinical instructor and the facility charge nurse immediately — not at the end of the shift.
3. Complete the facility incident report as directed by the charge nurse, and a GAHCTC incident report the same day.

4. Do not alter documentation, discard anything, or discuss the incident with anyone other than the instructor, the charge nurse, and facility management.
5. Participate in the post-conference debrief. Incidents are reviewed with the cohort in a de-identified manner so that all students learn from them.
 - Students follow the host facility's exposure control plan, to which they are oriented on Clinical Day 1.
 - Failure to report an incident is itself a violation and is treated more seriously than the underlying event.

13.7 Liability Insurance Requirement

Every student must carry student professional liability (malpractice) coverage in force for the entire clinical rotation. Coverage is arranged through GAHCTC at a cost of \$25.00 per student, payable with program fees.

- The \$25.00 liability insurance fee is required of all students and is separate from tuition. It is listed on the GAHCTC itemized cost schedule.
- Proof of coverage must be on file with the Program Director before the first clinical day. A student without documented coverage will not be permitted on the clinical unit and will be deferred to the next cohort, since clinical hours cannot be substituted.
- A student who carries individual professional liability coverage from another carrier may submit a current certificate of insurance for review; approval is at the discretion of the Program Director and does not automatically waive the fee.
- Student liability coverage applies to activities performed as a GAHCTC student within the nurse aide scope of practice and under supervision. It does not cover acts outside that scope, acts outside the program, or employment.
- GAHCTC does not provide health insurance or workers' compensation coverage for students. See Section 12.6.

SECTION 14: CERTIFICATION & STATE EXAMINATION PREPARATION

14.1 Georgia Nurse Aide Competency Evaluation — Overview

The final online module and the final skills laboratory session are dedicated to program review and state examination preparation. The Georgia competency evaluation is the NNAAP® examination, administered by Credentia under contract with Georgia DCH, and consists of a written (or oral) knowledge examination and a clinical skills evaluation. Both parts must be passed for registry listing.

- Written (or oral) examination format, item count, time limit, and passing standard, reviewed in the final online module.
- Clinical skills evaluation: evaluator criteria, critical elements, and automatic failure criteria, reviewed online and reinforced during the final skills laboratory session.
- Hand hygiene is always included among the skills selected for the skills evaluation; students practice it to mastery throughout the program.
- High-frequency tested skills are practiced during the skills laboratory using the same critical-step standard applied by state evaluators.
- How to communicate effectively with the evaluator during the skills examination.

Examination fees, item counts, time limits, retake limits, and the window within which a candidate must test after completing training are set by Credentia and Georgia DCH and change from time to time. The Program Director verifies the current requirements against the Credentia Georgia candidate handbook before each cohort's examination preparation session, and the current figures are distributed as a dated handout rather than printed in this manual.

14.2 Examination Registration

Students receive step-by-step instruction and a recorded walkthrough of the Credentia registration process for Georgia, including required identification, application submission, scheduling of the skills evaluation and the written or oral examination, testing center logistics, and rescheduling and cancellation rules. GAHCTC submits training completion documentation as required so that a graduate's eligibility can be verified.

14.3 The Georgia Nurse Aide Registry

On passing both parts of the competency evaluation, a candidate is listed on the Georgia Nurse Aide Registry, administered by Alliant Health Solutions for Georgia DCH. Students receive a handout covering:

- How to access and verify registry status.
- The expected timeline from examination to registry listing.
- Renewal requirements — a minimum of eight hours of paid nursing-related work within each 24-month period.
- Reciprocity for nurse aides certified in another state.
- Current registry contact information, distributed as a dated handout and verified before each cohort.

14.4 Professional Development and Advancement

Final module instruction includes résumé and job application guidance delivered online, interview preparation, and information on advancement pathways — medication aide, LPN, RN, and healthcare leadership — preparing graduates for continued professional growth beyond initial certification. GAHCTC continuing education offerings and certification courses are introduced as next steps.

SECTION 15: GRIEVANCE & APPEALS PROCEDURE

GAHCTC is committed to fair, transparent, and timely resolution of student concerns arising from online instruction, skills laboratory sessions, or clinical orientation. The following four-step process applies to all program participants.

1. Informal resolution — the student raises the concern directly with the instructor or party involved within five business days of the incident, by email, TalentLMS message, or in person.
2. Formal written grievance — if not resolved informally, the student submits a written grievance to the Program Director within ten business days, describing the concern and the resolution sought.
3. Program Director review — the Program Director reviews all documentation and issues a written decision within ten business days of receipt.
4. Appeal to the CEO — if unsatisfied, the student may submit a final written appeal to the CEO within five business days of the Program Director's decision. The CEO's decision is final within GAHCTC.

All grievance records are maintained confidentially for a minimum of three years. Students will not face retaliation for filing a good-faith grievance. Students who believe a concern has not been resolved may contact the Georgia Department of Community Health; contact information is provided on request and posted at the training facility. Grievances concerning the state competency evaluation itself are directed to Credentia under its published grievance process.

The facility grievance process and the role of the Long-Term Care Ombudsman in resident rights grievances are addressed separately in the Part VII online module and reinforced during the skills laboratory.

SECTION 16: PROGRAM QUALITY IMPROVEMENT

GAHCTC pursues continuous program improvement guided by data, including data specific to the blended delivery model. Quality improvement activities include:

- Annual curriculum review by the Program Director and qualified RN instructors, incorporating updates to the Hartman textbook edition and review of TalentLMS module effectiveness.
- Analysis of Georgia Nurse Aide Competency Evaluation pass rates, reported separately for the written and skills components — target: 80% first attempt.
- Analysis of online module completion rates, quiz and chapter test performance, item-level difficulty, time-to-completion, and attrition points within TalentLMS.
- Comparison of online assessment performance against in-person final examination and skills check-off performance, to confirm that online assessment is predicting in-person competence.
- Graduate employment tracking and annual employer satisfaction surveys.
- Post-clinical program survey administered at the end of Clinical Day 3.
- Student course evaluations at the conclusion of each cohort, including specific feedback on the online platform and the on-site check-in process.
- Clinical partner feedback collected at the end of each rotation.
- Annual program renewal documentation submitted to Georgia DCH, inclusive of the blended learning delivery model.
- Review and integration of updates to OBRA '87, CMS guidance, Georgia DCH rules, and Credentia examination requirements.
- Faculty development aligned with current evidence-based instructional practice, including online teaching methodology.

Program improvement plans are developed whenever a benchmark is not met and are reviewed by the Program Director and CEO quarterly.

STUDENT ACKNOWLEDGMENT OF RECEIPT

I acknowledge that I have received, read, and understand the GA Healthcare Training & Consulting Nurse Aide Training Program Policy & Procedure Manual, Blended Learning Track. I agree to abide by all policies, standards, and requirements set out in it as a condition of my participation, including the 98-hour blended curriculum structure — 40 online hours, 34 in-person skills laboratory hours, and 24 clinical orientation hours — and all on-site check-in and clinical practicum requirements.

I understand that all 98 approved program hours must be completed before a Certificate of Completion is issued and before I may be submitted as eligible for the Georgia Nurse Aide Competency Evaluation.

I understand that the comprehensive final examination is administered in person at the GAHCTC facility and cannot be taken online.

I understand that failure to comply with program policies may result in disciplinary action up to and including dismissal. I understand that this manual is subject to revision and that I will be notified in writing of any substantive change. I understand that specific module deadlines, skills laboratory check-in dates, and clinical orientation dates are published on the GAHCTC TalentLMS campus and are not included in this manual.

I understand that this program's approval status with the Georgia Department of Community Health is as stated in Section 1.6 of this manual, and that GAHCTC has made no representation to me beyond what is stated there.

Signature	Date
Student Printed Name	
Student Signature	
Program Director Signature	

GA Healthcare Training & Consulting

Yolaine Nozile, PhD, RN, CLNC, FCN — Founder & CEO

Building Skills. Changing Lives. Advancing Healthcare.

